ARKANSAS DEPARTMENT OF HUMAN SERVICES

Division of Provider Services & Quality Assurance
Office of Long-Term Care
P.O. Box 8059, Slot S404, Little Rock, AR 72203-1437

Notice of Serious Incident

Case Number: 031895

Date of Incident:

Date Received: 08/07/2025

Facility Name: Little Creek Behavioral Health

Incident Type: DPSQA - Complaint

Case Status: Under Review

DPSQA - Complaint

Report Description: AO is Little Creek Behavioral facility. It was reported that the facility is frequently out of ratio. When DHS comes to check in a staff member or the director sends out a text message to everyone to everyone telling them to get in ratio and then when DHS leave they go back to being out of ratio. There are multiple polices being broken, staff is allowing residents to use their phones even when they?re not allowed to. There are residents that fight in the bathrooms and staff wont do anything about it. There was reportedly a staff member who showed nudes to a resident but it is unknown if she still works there and reportedly the supervisors where aware and didn?t do anything. There are also previous employees who call some of the residents and tell them to riot. The staff are reportedly covering up a lot of things at the facility and no one is doing anything. The staff are able to turn off and on the cameras in the facility.

Arkansas State Agency

STATEMENT OF DEFICIENCIES AND PLAN OF CORRECTION	(X1) PROVIDER/SUPPLIER/CLIA IDENTIFICATION NUMBER:	(X2) MULTIPLE CONSTRUCTION A. BUILDING: B. WING	(X3) DATE SURVEY COMPLETED
	3018		08/12/2025

NAME OF PROVIDER OR SUPPLIER: LITTLE CREEK BEHAVIORAL HEALTH

STREET ADDRESS, CITY, STATE, ZIP CODE: 161 SKUNK HOLLOW ROAD, CONWAY, AR 72032

(X4) ID PREFIX TAG	SUMMARY STATEMENT OF DEFICIENCIES (EACH DEFICIENCY MUST BE PRECEDED BY FULL REGULATORY OR LSC IDENTIFYING INFORMATION)	ID PREFIX TAG	PROVIDER'S PLAN OF CORRECTION (EACH CORRECTIVE ACTION SHOULD BE CROSS-REFERENCED TO THE APPROPRIATE DEFICIENCY)	(X5) COMPLETE DATE
- 000	Initial Comment The facility was not in compliance with Title 20, Chapter 10. Office of Long Term Care, Division of Provider Services and Quality Assurance, Department of Human Services Part 415. Psychiatric Residential Treatment Facility Providers. Els 31895 and 31935 - not in compliance	- 000		
-501 SS=E	Use of restraints (a)(1) The United States Children ' s Health Act of 2000 (Public Law No. 106-310) imposes procedural reporting and training requirements regarding the use of restraints and involuntary seclusion in Psychiatric Residential Treatment Facilities that provide inpatient psychiatric services for children who are under twenty-one (21) years of age. (2) Satellite training by the Centers for Medicare & Medicaid Services may be accessed on the Centers for Medicare & Medicaid Services website at www.cms.gov. Federal regulations are located at 42 C.F.R. § 483, Subpart G (2001). (3) The use of mechanical restraints and seclusions are strictly prohibited. (b) A Psychiatric Residential Treatment Facility will report monthly on an approved Office of Long-Term Care template, to the State-designated Protection and Advocacy system, the Office of Long-Term Care, and the Resident ' s PASSE, or the Resident ' s sending state the following data: (1) Physical restraints used within the Psychiatric Residential Treatment Facility, in the aggregate and by client; (2) Chemical restraints used within the Psychiatric Residential Treatment	-501		

LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE: [signature] TITLE: Director of Nurse (X6) DATE: 9/26/25

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-501	Continued From page 1 Facility, in the aggregate and by client; (3) Sedating PRNs within the Psychiatric Residential Treatment Facility, in the aggregate and by client; and (4) Ratio of chemical restraints to sedating PRNs, in the aggregate and by client. (c) An order for restraint must not be written as a standing order or on an asneeded basis. (d) Restraint must not result in harm or injury to the Resident and must be used only: (1) To ensure the safety of the Resident or others during an emergency safety situation; and (2) Until the emergency safety situation has ceased, and the Resident's safety and the safety of others can be ensured, even if the restraint order has not expired. (e) Orders for restraint must be ordered by a physician, or other licensed practitioner permitted by the state and the Psychiatric Residential Treatment Facility to order restraint and trained in the use of emergency safety interventions. (f) If the Resident's treatment team physician is available, only he or she can order a restraint. (g) A physician or other licensed practitioner permitted by the state and the Psychiatric Residential Treatment Facility to order a restraint must order the least restrictive emergency safety intervention that is most likely to be effective in resolving the emergency safety situation based on consultation with staff. (h)(1) If the order for restraint is verbal, the verbal order must be received by a	-501		

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-501	Continued From page 2 registered nurse or other licensed staff such as a licensed practical nurse, while the emergency safety intervention is being initiated by staff or immediately after the emergency safety situation ends. (2) The physician or other licensed practitioner permitted by the state and the Psychiatric Residential Treatment Facility to order restraint must verify the verbal order in a signed written form in the Resident's record. (3) The physician or other licensed practitioner permitted by the state and the Psychiatric Residential Treatment Facility to order restraint must be available to staff for consultation, at least by telephone, throughout the period of the emergency safety intervention. (i) Each order for restraint must: (1) Be limited to no longer than the duration of the emergency safety situation; and (2) Under no circumstances exceed four (4) hours for Residents who are between eighteen (18) years of age and twenty-one (21) years of age; two (2) hours for Residents who are between nine (9) years of age and seventeen (17) years of age; or one (1) hour for Residents who are under nine (9) years of age. (j) Within one (1) hour of the initiation of the emergency safety intervention a physician, or other licensed practitioner trained in the use of emergency safety interventions and permitted by the state and the Psychiatric Residential Treatment Facility to assess the physical and psychological well-being of Residents, must conduct a face-to-face assessment of the physical and psychological well-being of the Resident,	-501		

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-501	Continued From page 3 including but not limited to: (1) The Resident's physical and psychological status; (2) The Resident's behavior; (3) The appropriateness of the intervention measures; and (4) Any complications resulting from the intervention. (k) Emergency safety intervention: An emergency safety intervention must be performed in a manner that is safe and proportionate and that is appropriate to the severity of the behavior and to the Resident's chronological and developmental age; size; gender; physical, medical, and psychiatric condition; and personal history (including any history of physical or sexual abuse). (l) Staff must document the intervention in the Resident's record. That documentation must be completed by the end of the shift in which the intervention occurs. If the intervention does not end during the shift in which it began, documentation must be completed during the shift in which it ends. Documentation must include all of the following: (1) Each order for restraint; (2) The time the emergency safety intervention actually began and ended; (3) The time and results of the one (1) hour physical and psychological assessment required via face-to-face of the well-being of the Resident as required above; (4) The emergency safety situation that required the Resident to be restrained; and (5) The name of staff involved in the emergency	-501		

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-501	Continued From page 4 safety intervention. (m) The Psychiatric Residential Treatment Facility must maintain a record of each emergency safety situation, the interventions used, and their outcomes. (n) The physician or other licensed practitioner permitted by the state and the Psychiatric Residential Treatment Facility to order restraint must sign the restraint order in the Resident's record as soon as possible. Authority. Arkansas Code §§ 20-76-201 et seq., and 25-10-129. This ELEMENT is not met as evidenced by: Based on file review and interview, the facility failed to ensure documentation of physical restraint for two (Client #3 and Client #4) of five clients. The findings include: A review of Client #3's and Client #4's file on 08/11/2025 did not reveal a Restraint Packet had been completed for the physical restraint conducted during the elopement on 08/08/2025. On 08/11/2025 at 12:21 PM, during an interview Client #3 stated that he had been caught and placed in a physical restraint by BHA #1 at the gas station where they were found after elopement. On 08/12/2025 at 11:05 AM, during an interview BHA #1 stated upon finding the clients who had eloped to a gas station he was able to catch Client #3 and placed him in a regular standing hold until the local police department arrived and took custody of Client #3. On 08/11/2025 at 11:51 AM, during an interview Client #4 stated that he hurt his ankle when he hit the ground after jumping out of the window so when staff arrived at the gas station he started to	-501		

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-501	Continued From page 5 run but gave himself up. On 08/12/2025 at 2:25 PM, during an interview BHAS #1 stated that upon finding the clients who had eloped at a gas station Client #4 attempted to run so he restrained him with a regular standing hold until the local police department arrived and took custody of Client #4. A review of the local Sheriff's Office incident report noted at the time of the author's arrival to the gas station where clients were found by unidentified facility staff two Sheriffs Office personnel were assisting the facility staff in placing the juveniles in hand restraints. On 08/12/2025 at 12:26 PM, during an interview the Chief Executive Officer acknowledged there was no documentation of the restraints for Client #3 and Client #4 and stated that if there was involvement from facility staff prior to the local police department taking possession of the clients then there should be documentation of it.	-501		
-502 SS=D	Notification of Parent or Legal Guardian (a) The Psychiatric Residential Treatment Facility must notify the parent or legal guardian of the Resident who has been restrained as soon as possible after the initiation of each emergency safety intervention. (b) The Psychiatric Residential Treatment Facility must also notify the Resident ' s parent or legal guardian as soon as possible and in no case later than twenty-four (24) hours after the emergency safety intervention. (c) The Psychiatric Residential Treatment Facility must document in the Resident's record that the parent or legal guardian has been notified of the emergency safety intervention, including the date and time of notification and the name of the staff	-502		

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-502	Continued From page 6 person providing the notification. Authority. Arkansas Code §§ 20-76-201 et seq., and 25-10-129. This ELEMENT is not met as evidenced by: Based on interview and record review, the facility failed to notify the guardian of a restraint for two (Client #3 and Client #4) of five clients reviewed. A review of the restraint documentation dated 08/08/2025 revealed Client #2 received a chemical restraint of Haldol, an antipsychotic medication and Benadryl, an antihistamine medication at 7:20 AM upon returning to the facility after an elopement for aggression toward the staff. The restraint documentation did not have evidence that the parent or guardian was notified. A review of the restraint documentation dated 08/08/2025 revealed Client #3 received a chemical restraint of Haldol and Benadryl 7:20 AM upon returning to the facility after an elopement for verbal aggression toward staff, peers and law enforcement. The documentation did not have evidence that the parent or guardian was notified. A review of restraint documentation dated 08/08/2025 revealed Client #4 received a chemical restraint of Haldol and Benadryl at 7:15 AM upon returning to the facility after an elopement for verbal aggression toward peers and law enforcement. The document did not have evidence that the parent or guardian was notified.	-502		